RHT across all 50 states

What states are proposing

States combine workforce and care-delivery changes with digital investment to expand rural access. The mix ranges from hospital and EMS redesign to regional care networks, prevention, maternal services, behavioral health, and technology modernization.

PROGRAM FOCUS

50 workforce activities
34 behavioral health activities

TECHNOLOGY FOCUS

40 telehealth / remote care
43 data / records

What leadership should take from this

1. Use the state profiles to identify comparable approaches and promising follow-up questions.
2. Separate proposed activities from financial obligations and operational services.
3. Test whether behavioral health providers actually participate in digital investments and shared clinical workflows.

Three different evidence dates

December 2025: state proposal abstracts underpin the heat map and summaries.
July 31, 2026: state-reported obligations provide a separate financial snapshot.
Through October 9, 2026: the supplied workbook indexes later announcements; these headlines do not establish operational status.

Alabama – California

Alabama

Alabama proposes regional IT and cybersecurity hubs, shared EHR infrastructure, and statewide telehealth networks, alongside maternal care, cancer screening, EMS, and workforce investments. Its behavioral health plans include converting community mental health centers to CCBHCs and developing clinics that integrate behavioral, physical, and dental care.

Abstract PDF p. 5 | December 2025 proposal

Alaska

Alaska proposes six community-led initiatives spanning maternal and child health, essential services, prevention, value-based care, workforce retention, and technology innovation. Its technology agenda includes telehealth, consumer devices, AI, cybersecurity, and interoperable data, while its access initiative includes behavioral and home- and community-based care.

Abstract PDF p. 6 | December 2025 proposal

Arizona

Arizona proposes workforce grants, targeted behavioral health, chronic disease and maternal health programs, and expanded telehealth and mobile care. Shared-service networks, subsidized EHR licenses, and interoperable data systems would support rural providers and reduce administrative burdens.

Abstract PDF p. 7 | December 2025 proposal

Arkansas

Arkansas proposes four linked initiatives: HEART for prevention and wellness, PACT for integrated care networks, RISE AR for workforce development, and THRIVE for telehealth and remote monitoring. The plan combines primary and specialty access, including behavioral health services, with EMS modernization, analytics, and value-based delivery models.

Abstract PDF pp. 8–9 | December 2025 proposal

California

California proposes regional hospital-centered networks to improve primary, maternity, chronic disease, and specialty care, supported by workforce pathways and shared technology services. Planned tools include EHR and HIE modernization, cybersecurity, remote prenatal monitoring, e-consults, and perinatal psychiatry access programs.

Abstract PDF pp. 10–11 | December 2025 proposal

Colorado – Georgia

Colorado

Colorado proposes locally driven partnerships among hospitals, clinics, public health, schools, businesses, and behavioral health providers to improve prevention and essential services. Its investments would combine workforce recruitment and retention with telehealth, monitoring tools, equipment, and shared data systems.

Abstract PDF p. 12 | December 2025 proposal

Connecticut

Connecticut proposes four coordinated initiatives covering population health, workforce, data and technology, and care transformation and stability. It would expand HIE participation, telehealth, and analytics while promoting coordinated teams and integration of medical, behavioral, dental, and long-term services.

Abstract PDF pp. 13–14 | December 2025 proposal

Delaware

Delaware proposes new community access points through mobile units, schools, libraries, and Hope Centers, alongside a rural workforce pipeline and a proposed medical school track. Its plan also includes HIE infrastructure, a technology catalyst fund, diabetes remote monitoring, Food is Medicine, and value-based care readiness.

Abstract PDF p. 15 | December 2025 proposal

Florida

Florida proposes rural and satellite clinics, mobile care, community paramedicine, regional telepsychiatry hubs, and pharmacy-based access, supported by clinical training pathways. HIE onboarding, telehealth, remote monitoring, nutrition programs, and value-based payment initiatives would support care coordination and provider sustainability.

Abstract PDF p. 16 | December 2025 proposal

Georgia

Georgia proposes preparing rural providers for the AHEAD model while strengthening behavioral health, emergency readiness, newborn services, nutrition, and connections to care. Workforce scholarships and rural recruitment would be paired with technology investments in electronic records, cybersecurity, robotics, and AI.

Abstract PDF pp. 17–18 | December 2025 proposal

Hawaii – Iowa

Hawaii

Hawaii proposes a Rural Health Information Network and a statewide telehealth network, paired with EMS, community paramedicine, mobile care, and behavioral health infrastructure. Workforce pathways, rural medical respite, and a value-based innovation and AHEAD readiness fund would support local access and sustainability.

Abstract PDF p. 19 | December 2025 proposal

Idaho

Idaho proposes technology modernization, community and home-based care, workforce incentives, and targeted chronic disease, behavioral health, and maternal-child health programs. Investments include interoperable EHRs, telehealth, AI, cybersecurity, EMS communications, mobile units, and facility and pharmacy improvements.

Abstract PDF pp. 20–21 | December 2025 proposal

Illinois

Illinois proposes hospital transformation, community care infrastructure, and disease prevention, alongside EMS, mobile healthcare, and virtual care technology. A rural education pipeline and expanded training for healthcare and support workers would address workforce shortages.

Abstract PDF p. 22 | December 2025 proposal

Indiana

Indiana's GROW plan proposes regional innovation grants alongside a statewide medical coordination center, closed-loop 211 referrals, and stronger interoperability networks. It would also expand telehealth and tele-consultation, improve emergency pediatric and obstetric readiness and cardiometabolic care, and grow physician, behavioral health, and paraprofessional capacity.

Abstract PDF pp. 23–24 | December 2025 proposal

Iowa

Iowa's Healthy Hometowns plan proposes health hubs, provider recruitment, cancer prevention and treatment, and co-located services with community health worker navigation. Statewide HIE and EMS-enabled mobile care would support transitions between settings, maternal transport, chronic disease management, and care in the home.

Abstract PDF p. 25 | December 2025 proposal

Kansas – Maryland

Kansas

Kansas proposes prevention, sustainable primary care, workforce development, and value-based care, supported by data sharing, AI, remote monitoring, and consumer technologies. Its behavioral health strategy explicitly links primary care, emergency departments, and nursing facilities, alongside Food Is Medicine and transportation innovations.

Abstract PDF p. 26 | December 2025 proposal

Kentucky

Kentucky proposes community chronic care hubs, telehealth-enabled maternal teams, behavioral crisis response and recovery, rural dental access, and integrated EMS and trauma care. Workforce expansion and interoperable technology would support these five care models, including EmPATH, mobile crisis, and telehealth services.

Abstract PDF p. 27 | December 2025 proposal

Louisiana

Louisiana proposes six initiatives spanning workforce and emergency capacity, technology modernization, value-based care, prevention, integrated care for high-needs populations, and capital support. A state-managed EHR, technology catalyst fund, digital tools, AI, and mobile care platforms would support collaboration among hospitals, clinics, behavioral health providers, and EMS.

Abstract PDF pp. 28–29 | December 2025 proposal

Maine

Maine proposes population health programs, workforce growth, technology-enabled access, and affordability and provider sustainability strategies. Its stated targets emphasize blood pressure control, fewer readmissions and delayed care, and stronger rural hospital operating margins, with limited detail on specific digital tools in the abstract.

Abstract PDF p. 30 | December 2025 proposal

Maryland

Maryland proposes immediate-impact and longer-term investments in workforce, primary and specialty care, school-based health, behavioral health, and food access. Telehealth, remote monitoring, and predictive AI alerts would support access and chronic disease management, while local food infrastructure would advance prevention.

Abstract PDF pp. 31–32 | December 2025 proposal

Massachusetts – Missouri

Massachusetts

Massachusetts proposes seven initiatives spanning population health, new care models, workforce, community prevention, EMS integration, technology connectivity, and facility modernization. The plan pairs clinical infrastructure and value-based care support with regional capacity building and minor renovations to expand access.

Abstract PDF pp. 33–34 | December 2025 proposal

Michigan

Michigan proposes regional partnerships, workforce recruitment, and integrated behavioral health access, with prevention and chronic disease training. A technology catalyst fund, expanded data exchange, telehealth, remote monitoring, and digital referral networks would connect residents with clinical and community resources.

Abstract PDF p. 35 | December 2025 proposal

Minnesota

Minnesota proposes cardiometabolic prevention, rural training pathways, new community access points, regional care models, and technology and financial viability investments. Grants would support telehealth, EMS treatment in place, infrastructure, and provider technical assistance; community mental health centers and CCBHCs are named as potential subrecipients.

Abstract PDF pp. 36–37 | December 2025 proposal

Mississippi

Mississippi proposes regional systems linking EMS, hospitals, public health, and social services, alongside workforce expansion and rural facility investment. EHR modernization, telehealth, cybersecurity, psychiatric emergency capacity, and innovative care pilots would support behavioral health access, chronic disease care, and post-discharge coordination.

Abstract PDF pp. 38–39 | December 2025 proposal

Missouri

Missouri's ToRCH Care proposes regional coordinating networks and local community hubs connecting physical health, behavioral health, and social services. An interoperable digital backbone, alternative payment models, workforce pipelines, and provider modernization would support local primary and maternity care with specialty connections.

Abstract PDF p. 40 | December 2025 proposal

Montana – New Jersey

Montana

Montana proposes five initiatives focused on workforce, facility sustainability, innovative delivery and payment models, prevention, and modern health technologies. Its abstract emphasizes partnerships and restructuring but provides limited detail on specific technologies or clinical service lines.

Abstract PDF p. 41 | December 2025 proposal

Nebraska

Nebraska proposes Food as Medicine, regional access and navigation networks, workforce development, mobile and remote care, and a technology catalyst fund. Distinct initiatives address emergency behavioral health and substance use services, maternal and post-acute access, and assisted living for people with complex needs.

Abstract PDF pp. 42–43 | December 2025 proposal

Nevada

Nevada proposes an outcomes accelerator, a provider infrastructure flex fund, workforce recruitment and training, and an innovation and technology grant program. Priorities include value-based chronic disease care, remote and hybrid services, mobile and emergency capacity, shared purchasing, modern health records, and cybersecurity.

Abstract PDF p. 44 | December 2025 proposal

New Hampshire

New Hampshire proposes population health, rural access, workforce, technology, and financial solvency initiatives, including statewide CCBHC expansion and a dual-eligible special needs plan. Telehealth, AI, interoperability, cybersecurity, rural training, and value-based payments would support prevention, coordinated care, and long-term services.

Abstract PDF pp. 45–46 | December 2025 proposal

New Jersey

New Jersey proposes workforce investment and support for essential CCBHC, FQHC, and hospital providers, combining directed and competitive funding. Regional telehealth, remote monitoring, mobile care, community prevention, and chronic disease data integration would extend care beyond traditional offices.

Abstract PDF p. 47 | December 2025 proposal

New Mexico – Ohio

New Mexico

New Mexico proposes regional specialty and maternal care networks, rural and Tribal workforce pathways, and grants for locally designed health initiatives, including behavioral health. A sustainability and innovation center and statewide health analytics platform would support providers, integrate data, and strengthen planning and care delivery.

Abstract PDF pp. 48–49 | December 2025 proposal

New York

New York proposes rural health partnerships, technology-enhanced primary care, workforce development, and technology and cybersecurity investments. Telehealth, e-consults, and more usable alerts would support coordinated access across the care continuum, although the abstract provides little detail on behavioral health-specific activities.

Abstract PDF p. 50 | December 2025 proposal

North Carolina

North Carolina proposes NC ROOTS hubs that connect medical, behavioral, and social services, alongside prevention, maternal health, CCBHC and SUD service expansion, and rural workforce development. HIE participation, digital literacy, AI tools, and value-based payment readiness would support integrated regional care and provider sustainability.

Abstract PDF pp. 51–52 | December 2025 proposal

North Dakota

North Dakota proposes workforce stabilization, nutrition and physical activity programs, behavioral health prevention, and care closer to home through telehealth hubs, mobile clinics, and remote monitoring. Shared technology, data modernization, AI-assisted care, cooperative purchasing, and innovative payment models would support coordination and financial sustainability.

Abstract PDF pp. 53–54 | December 2025 proposal

Ohio

Ohio proposes regional innovation hubs, EMS treatment in place, school-based primary and behavioral health services, mobile vision, hearing and dental care, and a rural workforce pipeline. Telehealth, remote monitoring, interoperable records, and value-based financing would support maternal access and coordinated care across rural counties.

Abstract PDF pp. 55–56 | December 2025 proposal

Oklahoma – South Carolina

Oklahoma

Oklahoma proposes six initiatives covering innovative care, prevention, regional collaboration, value-based care, workforce development, and a health data utility. Its abstract sets out a broad transformation framework but offers limited detail on specific clinical services or digital tools.

Abstract PDF p. 57 | December 2025 proposal

Oregon

Oregon proposes regional partnerships, prevention, workforce resilience, technology and data modernization, and a dedicated Tribal initiative. Funding would combine near-term community-directed projects with longer-term regional ventures, including a set-aside for the nine federally recognized Tribes.

Abstract PDF p. 58 | December 2025 proposal

Pennsylvania

Pennsylvania proposes eight regional Rural Care Collaboratives to strengthen shared services, telehealth, clinical integration, and access to primary and specialty care. Workforce scholarships and training, data and analytic support, and alternative payment approaches would address locally identified priorities including maternity, behavioral health, dental care, and aging.

Abstract PDF p. 59 | December 2025 proposal

Rhode Island

Rhode Island proposes community clinical hubs, mobile care, EMS expansion, hospital-at-home, and stronger behavioral health crisis and recovery services. A state-sponsored EHR, data systems, workforce programs, and value-based payment support would connect services for rural, island, Tribal, older adult, and disability populations.

Abstract PDF pp. 60–61 | December 2025 proposal

South Carolina

South Carolina proposes EHRs, telehealth, remote monitoring, and a statewide resource platform, combined with chronic disease and pediatric care improvements. Mobile units, crisis response teams, workforce and facility investments, and a technology catalyst fund would expand local access and support maternal health and long-term sustainability.

Abstract PDF p. 62 | December 2025 proposal

South Dakota – Vermont

South Dakota

South Dakota proposes EHR and HIE expansion, telehealth, cybersecurity, and a Rural Data Atlas alongside workforce incentives and facility optimization. Its care strategy includes statewide CCBHC implementation, behavioral health integration through the Collaborative Care Model, regional EMS hubs, maternal care, and new payment approaches.

Abstract PDF p. 63 | December 2025 proposal

Tennessee

Tennessee proposes rural facility transformation, maternal and child health, prevention and nutrition, technological infrastructure, and workforce pipelines. Competitive grants and outcomes-based payment approaches would support local care delivery, with an additional shared-savings capital investment described separately from RHT funds.

Abstract PDF p. 64 | December 2025 proposal

Texas

Texas proposes wellness and nutrition programs, consumer-facing tools, AI and telehealth, workforce development, unified care infrastructure, cybersecurity, and capital investment. Shared technology and collaboration would support care coordination, stronger rural provider capacity, and chronic disease prevention.

Abstract PDF p. 65 | December 2025 proposal

Utah

Utah proposes seven initiatives covering prevention, workforce incentives, care infrastructure, financial sustainability, telehealth, shared operational resources, and interoperability. Its LINCS initiative explicitly aims to connect data across clinics, hospitals, behavioral health providers, and community organizations to enable coordinated care.

Abstract PDF p. 66 | December 2025 proposal

Vermont

Vermont proposes regionalizing services and creating a clinically integrated network of shared services, alongside stronger team-based primary care for chronic disease. Workforce and housing investments, patient-facing technologies, and price transparency and insurance competition tools would address access, staffing, and affordability.

Abstract PDF pp. 67–68 | December 2025 proposal

Virginia – Wyoming

Virginia

Virginia's Rural Vitality plan proposes health technology investment, rural training pipelines, prevention, and expanded local access. CareIQ would support EHRs, productivity tools, startups, and remote monitoring, while mobile clinics, telehealth hubs, community paramedicine, and maternal networks would connect care closer to home.

Abstract PDF p. 69 | December 2025 proposal

Washington

Washington proposes hospital innovation, community prevention, Native family health, workforce development, technology adoption, and expansion of rural behavioral health. AI and cybersecurity investments, training for Tribal and long-term care workers, and payment transformation for hospitals and behavioral health clinics would support sustainability.

Abstract PDF p. 70 | December 2025 proposal

West Virginia

West Virginia proposes a connected care grid, coordinated transportation, workforce development, payment and data tools, prevention, and health technology innovation. A Health to Prosperity Pipeline would link recovery and care management with employment, while telehealth and remote monitoring extend local access.

Abstract PDF p. 71 | December 2025 proposal

Wisconsin

Wisconsin proposes workforce grants and career pathways, technology and digital infrastructure upgrades, and regional partnerships to create coordinated systems of care. Telehealth, closed-loop referrals, and a digital rural health collaborative would help connect primary, specialty, behavioral health, prevention, and community supports.

Abstract PDF pp. 72–73 | December 2025 proposal

Wyoming

Wyoming proposes stronger rural hospitals and ambulance systems, integrated primary and behavioral care, workforce pipelines, and prevention. Statewide telepsychiatry and crisis services, care and transportation coordination, and technology and payment changes would support chronic disease management and care closer to home.

Abstract PDF p. 74 | December 2025 proposal

Specific technology focus

Technology is common, but the abstracts vary sharply in specificity.

The counts below require a named technology. Generic references to innovation, modernization, or technology infrastructure do not qualify.

Telehealth / virtual care / e-consults 37

AL, AK, AZ, AR, CA, CO, CT, DE, FL, HI, ID, IL +25 more

Remote patient monitoring 15

AL, AR, DE, FL, KS, MD, MI, NE, NJ, ND, OH, RI +3 more

Electronic health records (EHR / EMR) 13

AL, AZ, CA, GA, HI, ID, LA, MS, OH, RI, SC, SD +1 more

HIE / interoperability / data exchange 24

AK, AZ, CA, CT, DE, FL, HI, ID, IN, IA, KS, KY +12 more

AI / robotics 12

AL, AK, GA, ID, KS, LA, MD, NH, NC, ND, TX, WA

Cybersecurity / cyber protection 12

AL, AK, CA, GA, ID, MS, NV, NH, NY, SD, TX, WA

Digital or closed-loop referrals 3

IN, MI, WI

Consumer devices / wearables 6

AK, KS, MS, NE, TX, VT

These overlapping categories describe proposed tools, not procurement, implementation, use, or outcomes. State-level details and source pages are in the evidence inventory.

Technology mentioned by care area

Technology counts only when the abstract mentions it for the care area.

Behavioral health + technology

10 technology mentioned

24 care area mentioned; no technology mentioned

Home / LTC + technology

3 technology mentioned

8 care area mentioned; no technology mentioned

Technology mentioned for behavioral health

Arizona, Florida, Kentucky, Missouri, New Hampshire, North Carolina, Ohio, Rhode Island, Utah, Wyoming

Technology mentioned for home / long-term care

Idaho, Iowa, Rhode Island

HOW TO READ THE MAPS

- Technology mentioned for this care area
- Care area mentioned; no technology mentioned
- Care area not mentioned in abstract

These describe proposal abstracts, not implementation. See the Care-area evidence sheet for the state-specific rationale and source pages.

Technology and behavioral health

The key question is whether digital investment connects providers and changes care delivery.

The internal project brief asks how RHT investments include specialty behavioral health across primary care, emergency/crisis care, long-term care, and community settings. The supplied abstracts identify useful designs, but rarely establish participation or functioning workflows.

Utah | data connections

LINCS explicitly names data sharing across clinics, hospitals, behavioral health providers, and community organizations. Verify onboarding, consent, exchanged data, and actual use. [Abstract PDF pp. 66–67]

Rhode Island | connected care hubs

Community clinical hubs link behavioral health professionals and community organizations through interoperable technology; other initiatives include crisis services and hospital-at-home. Verify whether these initiatives share referrals and follow-up. [pp. 60–61]

Kansas | care-setting integration

The proposal embeds behavioral health in primary care, emergency departments, and nursing facilities. Verify how technology supports those connections and which providers participate. [p. 26]

South Dakota | BH and digital investments

CCBHCs and Collaborative Care appear alongside EHR, HIE, and telehealth plans. Their coexistence does not establish BH provider access to the funded technology. [p. 63]

These are illustrative follow-up candidates, not a ranking or a finding of implementation.

How to read and use this package

Scope and coding

All 50 states were reviewed using the supplied December 2025 abstract compilation. Categories are overlapping, analyst-defined descriptions of activities mentioned in the abstracts. A gray cell means the activity is not mentioned in this abstract, not that the state lacks it. Broad technology or sustainability language does not establish a specific technology or payment model.

Evidence and review status

Two-sentence summaries are analyst syntheses, not state quotations. This is a first-pass document review without independent second-coder validation. The Excel inventory includes category definitions, supporting text windows, full extracted state abstracts, page citations, and file hashes. The internal brief's preliminary BH mention/integration counts use a different concept from this activity-based heat map.

Financial interpretation

The obligations report covers December 29, 2025–July 31, 2026 (PDF p. 1). The workbook reproduces state-header awards and "Total Funds Put to Work," without treating them as expenditures or implementation. Abstract budgets are illustrative (abstract PDF p. 2). Later headline amounts are not added to obligations or used to rate progress.

Program guidance

The provider-payments fact sheet distinguishes services from infrastructure and describes a 15% Category B cap (p. 1). The capital fact sheet describes a 20% Category J cap and bars construction (p. 1). Workforce guidance distinguishes individual clinical recruitment/retention benefits from broader system supports; the five-year service requirement depends on the activity. These rules constrain interpretation of proposed investments, not state performance.

Additional context

The Rural Tech Catalyst Fund guide describes an optional fund for novel consumer-facing technology for chronic disease prevention and management (p. 1); it is not synonymous with all IT spending. The July 2026 Rural Health Resource Guide is a resource directory, not evidence that states implemented listed programs. Internal Word briefs frame the BH integration question; the press-release workbook contains headlines, not verified release bodies.

Package navigation

Start with the five-page heat map PDF or the offline interactive dashboard. Use the state digest for brief descriptions and the Excel inventory for citations, definitions, care-area evidence, funding context, and source text. Keep the original intake folder adjacent to deliverables for dashboard source links. No external sources were used.